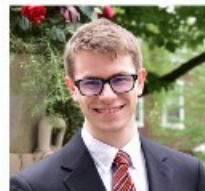




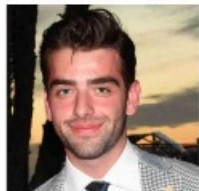
ChemoMicrobiome



Kai Trepka
Inventor



Chiara Marley
Regulatory Strategy



Thomas Settlemayer
Intellectual Property



Sparshika Chhaperia
Clinician



Abhay Bhat
Business



Allegra Messinese
Fundraising

Number of interviews since last meeting: **17**
Total number of interviews: **24**

ChemoMicrobiome provides oncology clinics with a clinical decision-support tool that uses ***patient-specific*** microbiome data to predict ***chemotherapy side effects***—driving better care and lower costs.



ChemoMicrobiome

Expected chemotherapy side effects:

Diarrhea: 10%

Nerve pain: 90%

Discontinue therapy: 10%



Customers: Who Matters?



Stakeholder Role	Key Player	Info/Value Flow
Buyer	Pharma, Hospitals	Buy diagnostic tools for clinical trials or patient care
End-User	Oncologists	Use results to inform treatment plans
Influencer	Patients, Academic KOLs	Push for more personalized care and testing
Recommender	Patient Advocacy Groups	Promote acceptance
Saboteur	Insurers, Regulatory Risk	Resist without clear clinical/economic proof

Key Learning 1- Oncologists

What Oncologists Really Want: Survival First



Here's what we thought:

- Oncologists would welcome toxicity prediction tools.

Here's what we did:

- Interviewed 8 oncologists (hematology, GI, prostate, breast).



What we learned:

- Cancer control > side effect control
- Interest in microbiome, but only if it impacts survival.

Next step:

- Position diagnostic to show both survival and toxicity benefits.



TOXICITY

EFFICACY

Key Learning 2 – Patients

Patients: Hungry for Predictive Power

Here's what we thought:

- Patients might resist extra testing.

Here's what we did:

- Interviewed 2 patients with recent severe side effects.

What we learned:

- Patients WANT predictive tests — especially non-invasive ones (e.g., stool tests).
- Desire for more control over outcomes.

Next step:

- Build patient-centered messaging:
→ "Know before you start."



Key Learning 3 – Microbiome Companies

From Enthusiasm to Execution: Microbiome Company Reality

Here's what we thought:

- Diagnostics would be the easy commercialization route.

Here's what we did:

- Interviewed 9 microbiome founders and executives.

What we learned:

- Diagnostics = Harder than expected: sample variability, regulatory hurdles, payer resistance.
- Therapeutics = Long-term goldmine.

Next step:

- Design validation plan for diagnostics.
- Keep optional path open for microbiome therapeutic pivot.



Key Learning 4 – Pharma

Pharma's Reluctant Curiosity

Here's what we thought:

- Pharma would integrate biomarkers quickly.

Here's what we did:

- Interviewed 2 pharma translational scientists.

What we learned:

- Microbiome interest exists but adoption lags without validated biomarkers.
- Metadata inconsistency is a big concern.

Next step:

- Focus on generating clinical-grade, regulatory-grade validation data.



Key Learning 5 – Reimbursement

The Cost Question: Payers Need Proof

Here's what we thought:

- Validation = Reimbursement.

Here's what we did:

- Interviewed 1 insurance advisor.

What we learned:

- Cost-effectiveness case is essential.
- Need to show hospital savings and better patient outcomes.

Next step:

- Initiate early health economics modeling.



Surprises & Insights

Biggest A-ha Moments

Survival > Toxicity: Oncologists will tolerate toxicity for better survival odds.



Patients > Physicians: Patients are more eager than doctors for predictive testing.



Diagnostics = Harder: Regulation, reproducibility, reimbursement hurdles loom large.

Therapeutics = Horizon: Microbiome therapeutics seen as ultimate value play.

Original LSVC/BMC Vs. Current LSVC/BMC

Day 1 Assumptions	Reality Check
Primary Buyer: Oncologists	Primary Buyer: Pharma (clinical trials), Hospitals (later)
Key Value: Toxicity prediction	Key Value: Survival + Toxicity management
Primary Model: Standalone diagnostic	Model: Diagnostic first → Potential Therapeutic pivot
Expected Adoption: High and easy	Strategy: Early validation → Pharma partnerships → Payer engagement

What's Next: Pivot and Persevere

Build clinical validation study (pilot cohort)

Engage pharma translational teams

Develop health economic models for payers

Explore optional future pivot: microbiome therapeutics

